A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

22 April 2026

Dr Cornelius Ferwerda
Kaizen Clinics

Dear Dr Ferwerda,

Re: Mr Tuppahige Jayanithie
DOB: 1946-12-16
Address: 2/622 Warrigal Road, OAKLEIGH SOUTH, VIC, 3167

Summary

- **Gastroscopy revealing abnormal small intestinal mucosa - idiopathic sprue**
- **Biopsies negative for intestinal involvement of lymphoma.**
- **Review and surveillance gastroscopy in 12 months.**
- **No action required by you.**

I had the pleasure of conducting a review with Don following his recent gastroscopy. As you are aware, Don is currently being monitored for lymphoma. He recently returned from Sri Lanka where he noted some weight loss associated with loose stools, though his weight had remained relatively stable immediately prior to his recent procedure. The gastroscopy revealed an ongoing idiopathic sprue of the small intestinal mucosa. I have consulted with the Professor of Pathology regarding these findings and additional testing was performed to exclude the possibility of lymphoma involving the intestinal lining. I am pleased to report that the results are benign, with no evidence of malignancy or lymphomatous involvement identified. Despite this extensive histological review, the specific cause of the mucosal abnormality remains unclear. Following his gastroscopy and an iron infusion, Don experienced an episode of diarrhoea lasting approximately six days. While he speculated that this may have been secondary to a viral infection or a dietary indiscretion involving dairy, the duration of the symptoms was somewhat atypical for a simple reaction to milk.

Impression and Plan

Don will continue with his current medication regimen and I have provided him with a new prescription for budesonide. He remains under the care of the lymphoma clinic at Box Hill Hospital,

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Endoscopy locations
Epworth Freemasons Day Proc. Centre
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Hobson Healthcare Sydenham
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St Vincent's Private Hospital
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with his next review scheduled in six months. I have provided Don with a copy of his biopsy results and this correspondence to ensure his hospital records can be updated accordingly, as he noted some previous issues with the transfer of external documentation. We will touch base in 12 months to perform a surveillance gastroscopy to monitor his small intestinal mucosa.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

JAYANITHIE, TUPPAHIGE DON : SVP 852062, EHP 2643375

DOB: 16/12/1946 Age: 79 years

Unit 2, 622 Warrigal Rd, Oakleigh South 3167

Results from 11/04/2026 ..... Episode# 26P258058 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B009468 **CLINICAL NOTES** Atypical sprue. Gas = gastritis. See Penny's Previous report. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Gastric bx". Two mucosal fragments measuring 2-3 mm. A1. 2. Specimen labelled "D1 bx". Seven mucosal fragments measuring 1-7 mm. A1. 3. Specimen labelled "D2 bx". Six mucosal fragments measuring 2-5 mm. A1 **MICROSCOPIC DESCRIPTION And DIAGNOSIS** **1. Gastric biopsy: Fragments of antral type mucosa and body type mucosa with moderate chronic gastritis with some pyloric metaplasia but no Helicobacter in the surface mucus but no evidence of active inflammation, intestinal metaplasia or dysplasia. There is mild glandular atrophy. H. pylori immunohistochemistry is negative. 2. D1 biopsy: Fragments of flat mucosa with a dense lymphoplasmacytic infiltrate in the lamina propria with scattered eosinophils and neutrophils. There is only patchy increase in numbers of intraepithelial T-cells. Immunohistochemistry shows that there are some deep infiltrates of small B-cells as well as**

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infiltrates of small T-cells (CD2+, CD3+, CD5+, CD7+, with more CD4+ than CD8+). CD30 is negative. Kappa and lambda ISH does not show light chain restriction in the plasma cells. BCL6 shows some dispersed reactivity without nodule formation. CD21 shows very minor FDC meshwork formation. COMMENT: there is no evidence of follicular lymphoma or indolent T-cell lymphoproliferative disorder of the GI tract. The etiology of the flat mucosa and increased lymphoplasmacytic infiltration is unclear. Angiotensin II receptor blocker injury usually resolves after cessation of the drug. Other drugs such as immune checkpoint inhibitors, tropical sprue, protein intolerance can cause similar features. 3. D2: Fragments of flat mucosa similar to that seen in D1. Pathologist: A/Prof. P.A. McKelvie

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